Test Document

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TABLE I
Comparative Summary of Prior Studies on Machine Learning for CKD Prediction vs. This Work

Study	Cohort	Target	Validation	Leakage Status	Key Findings / Methodological Limitations
Almansour et al. [8]	UCI CKD (400)	Binary	70/30 Holdout	Critical (S_{cr} kept)	97.5% Acc. via ANN; cannot stage; trivial threshold rule.
Senan et al. [9]	UCI CKD (400)	Binary	10-fold CV	Global Preproc.	98.2% Acc. via SVM; imputation executed prior to split.
Debal & Sitote [10]	UCI CKD (400)	Binary	5-fold CV	Critical (S_{cr} kept)	99.1% Acc. using RF; exploited direct creatinine cutoff.
Chen et al. [11]	EHR ($N = 1,200$)	5-Stage	10-fold CV	Partial (S_{cr} kept)	82.4% Acc.; predicted eGFR stages with creatinine as input.
Ravizza et al. [12]	EHR ($N = 10,450$)	Decline	Patient-level	Leakage-Free	C-index 0.79 for rapid progression; requires EHR history.
Schuld et al. [13]	Synthetic	Binary	Holdout	Synthetic Data	Formalized quantum kernel embedding; no clinical test.
Havlicek et al. [14]	Supercond. QPU	2-Qubit	Holdout	Synthetic Data	Demonstrated ZZ-feature map kernel on toy classification.
This Work	UCI ($N = 374$)	5- & 3-Class	Nested 5×5	Strictly Leak-Free	0.8204 / 0.8563 QWK; quantum negative result; SHAP.

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